

--SUMMARY--

Decision No. 1642/15

20-Aug-2015

W.Sutton

- Recurrences (compensable injury)

No Summary Available

19 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w4015n]

Style of Cause:

Neutral Citation: 2015 ONWSIAT 1843



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 1642/15

BEFORE:

W. Sutton: Vice-Chair

HEARING:

August 6, 2015 at Toronto
Oral
No post-hearing activity

DATE OF DECISION:

August 20, 2015

NEUTRAL CITATION:

2015 ONWSIAT 1843

DECISION UNDER APPEAL:

WSIB Appeals Resolution Officer (ARO) D. M. Sheppard, dated
June 20, 2013

APPEARANCES:

For the worker:

T. Ross, Office of the Worker Adviser

For the employer:

Not participating

Interpreter:

Not applicable

**Workplace Safety and Insurance
Appeals Tribunal**

505 University Avenue 7th Floor
Toronto ON M5G 2P2

**Tribunal d'appel de la sécurité professionnelle
et de l'assurance contre les accidents du travail**

505, avenue University, 7^e étage
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REASONS

(i) Introduction

- [1] The worker appeals a decision of the ARO, which concluded that the worker did not have ongoing entitlement for his right shoulder, including claimed recurrences and a permanent impairment and non-economic loss (NEL) award for the right shoulder. The ARO rendered the decision based on the written record, without an oral hearing.

(ii) Issues

- [2] The issues under appeal are as follow:
1. Did the worker recover by November 1998 from a compensable shoulder injury of August 26, 1998?
 2. Does the worker have ongoing entitlement beyond November 1998, including the recurrences of December 13, 1999, March 30, 2001 and October 19, 2007?
 3. Does the worker have entitlement to a permanent impairment for the right shoulder?

(iii) Background

- [3] The following are the basic facts.

- [4] The now 38-year-old worker commenced his employment as a security officer with the accident employer, a shopping centre management company, in December 1996. On August 26, 1998, the worker claimed to have injured his right shoulder. At the time of this injury, the worker was 22 years of age. On his Form 6 Report of Injury/Disease, the worker stated: "While attempting to effect an arrest the suspect and I fell to the ground causing me to injure my right shoulder." The worker attended the emergency room where he was diagnosed with an anterior dislocation of the right shoulder joint. A shoulder reduction was performed and the worker underwent physiotherapy. Initial entitlement was granted for the injury.

- [5] On his Form 6 of January 4, 2000, the worker reported that since his injury "... his shoulder had not been the same, always a chronic pain. On Dec 13/99, while I was doing an aerobics exercise my shoulder became dislocated again. This time very little to no trauma occurred to cause my injury." The emergency room record reported that the worker had been performing a punch while participating in kick boxing.

- [6] On his Form 6 of May 4, 2001, the worker stated: "Previously two dislocations of the same shoulder ... This time I lost my balance on a set of stairs, my elbow hit the stairwell railing, resulting in the dislocation of the shoulder (right). Because of dislocation I fell down the remaining set of stairs." The worker was at the time working for a municipality as a licensing and standards officer. On May 17, 2001, the worker underwent an arthroscopic Bankart repair for the dislocation. He was granted entitlement for the recurrence of his injury and for his surgery.

- [7] On a Form 6 dated October 30, 2007, the worker stated: "I have had this injury from 1998-08-26 while at work arresting a person. It has occurred 2 other times, the last being in 2001 when I had surgery. This time I was off duty and I slipped getting up from ground and fell lightly

on my right arm, dislocating my shoulder.” At the time, the worker was employed with a transportation company as a special constable. The worker was not granted entitlement for this recurrence on the basis that it did not occur at work.

[8] On July 22, 2008, the Claims Adjudicator (CA) advised the worker as follows:

After reviewing your file I noted that you were previously granted entitlement to recurrences for right shoulder dislocations that occurred on December 13, 1999 and March 30, 2001. Loss of earnings (LOE) benefits were paid to you for lost time from work and healthcare benefits were also paid.

The information received to file confirms that on December 13, 1999 you were engaged in kick boxing when you received a punch and your right shoulder was dislocated. On March 30, 2001, you had another accident when you fell down the stairs at home and dislocated your right shoulder again.

The information on file supports that the accidents on December 13, 1999 and March 30, 2001, resulted from non work-related causes. Please be advised that the WSIB does not accept responsibility for these accidents.

According to the medical information on file, you had fully recovered from your compensable right shoulder dislocation of August 26, 1998 by November 26, 1998. Consequently, your work-related impairment was at complete recovery at the time of the first non work-related accident of December 13, 1999. As such there is no entitlement to benefits for an aggravation of a work-related injury. There is no entitlement to benefits for your right shoulder after November 26, 1998. Therefore there is no entitlement to benefits that you sustained to your right shoulder from the accidents that occurred on December 13, 1998, March 30, 2001 and October 19, 2007.

Based on my review and taking into account WSIB policy. I find that entitlement should not have been granted to you for the non-work related accidents of December 13, 1999 and March 30, 2001 and I am revoking those decisions. Entitlement for your non-work related accident of October 19, 2007 is also denied.

[9] In reaching a decision on the worker’s objection, the ARO concluded that the family doctor’s opinion that the second dislocation of 1999 was due to the initial injury in 1998 was not based on sound medical opinion and that following the compensable injury of August 1998, the worker had recovered from the injury in November 1998. The ARO also concluded that the 1999 injury was a significant new injury that was noncompensable. As such entitlement following November 1998 for the subsequent shoulder dislocations was denied.

(iv) Law and policy

[10] Since the worker was injured in August 1998, the *Workplace Safety and Insurance Act, 1997* (WSIA) is applicable to this appeal. All statutory references in this decision are to the WSIA, as amended, unless otherwise stated.

[11] Section 46 of the WSIA provides that if a worker’s injury results in permanent impairment, the worker is entitled to compensation for non-economic loss.

[12] “Impairment” means a physical or functional abnormality or loss (including disfigurement) which results from an injury and any psychological damage arising from the abnormality or loss.

[13] “Permanent impairment” means impairment that continues to exist after the worker reaches maximum medical recovery.

[14] Legislation and Board policy provide that the degree of a worker's permanent impairment is determined in accordance with the prescribed rating schedule or criteria, any medical assessments, and having regard to the health information on file. The prescribed rating schedule for most impairments is the American Medical Association's *Guides to the Evaluation of Permanent Impairment*, 3rd edition (revised) (the AMA Guides).

[15] Tribunal jurisprudence applies the test of significant contribution to questions of causation. A significant contributing factor is one of considerable effect or importance. It need not be the sole contributing factor. See, for example, *Decision No. 280*. The standard of proof in workers' compensation proceedings is the balance of probabilities.

[16] Pursuant to section 126 of the WSIA, the Board stated that the following policy packages, Revision No. 9, would apply to the subject matter of this appeal:

- Policy Package No. 38, "Recurrences,"
- Policy Package No. 40, "Maximum Medical Recovery (MMR)," and
- Policy Package No. 300, "Decision Making/Benefit of Doubt/Merits and Justice."

[17] I have considered these policies as necessary in deciding the issues in this appeal.

(v) Relevant medical evidence

[18] While I have reviewed the medical evidence in the worker's file in its entirety, I note, in particular, the following relevant medical reporting:

- August 26, 1998: X-rays of the worker's right shoulder identified an anterior dislocation of the right shoulder, which was confirmed as successfully reduced by imaging the following day.
- September 3, 1998: Orthopaedic surgeon, Dr. Brock, reported that the worker had suffered an acute dislocation of the right shoulder and was to undergo six weeks of immobilization, followed by physiotherapy.
- October 9, 1998: The worker's physiotherapist reported that the worker continued to experience decreased range of motion (ROM) in the shoulder along with decreased strength.
- October 15, 1998: Dr. Sedran, the worker's family physician, advised that the worker was recovering from a right shoulder dislocation and continued to experience right shoulder pain, limited ROM and inability to fully extend his right elbow.
- November 10, 1998: Dr. Sedran reported that, on assessment, the worker continued to have limitation in his shoulder movement and some shoulder apprehension or instability. Further physiotherapy for three to four weeks was recommended. Dr. Sedran stated: "I do anticipate a full recovery."
- January 12, 1999: The worker's physiotherapist completed a report for the Criminal Injuries Review Board, in which it was stated that "[d]uring the course of seven weeks therapy, [the worker's] range of motion returned to near full. Slight pain was still experienced at the end of range/strength but it was felt this would resolve with normal use."

The physiotherapist defined the worker's period of disability as August 26, 1998 to November 26, 1998.

- December 13, 1999: An emergency room record stated that the worker had dislocated his right shoulder "kick boxing – punched and dislocated." Noted was the worker's history of right shoulder dislocation.
- December 14, 1999: Dr. Sedran reported that the worker dislocated his right shoulder after work. He characterized the injury as "isolated injury; recurrent dislocations 13/12/99." The diagnosis was "recurrent shoulder dislocation."
- December 16, 1999: Dr. Chan, and orthopaedic surgeon, reported:

... Last week he was at the gym and was simply throwing a punch with no contact and dislocated the shoulder again. Once again he had to have the shoulder reduced ...

Examination shows this man with a bit of apprehension of his right shoulder to movement. There is slight diminished sensation over the axillary nerve distribution. He has muscle spasms with anterior subacromial tenderness.

This man therefore has a second episode of dislocation with suggestion of some axillary nerve damage. In his age group he most likely require [sic] surgical reconstruction to prevent further movement.
- January 5, 2000: In a Progress Report, Dr. Chan provided a diagnosis of "recurrent shoulder dislocation."
- January 14, 2000: The worker's physiotherapist reported: "Previous dislocation [therefore] more prone to recurrence.
- January 18, 2000: Dr. Sedran reported that the worker experienced a "minor abduction strain" that resulted in a right shoulder dislocation. He diagnosed "recurrent shoulder dislocation" with "shoulder apprehension, reduced range of motion." The worker was referred to a specialist for consideration of surgical management.
- January 21, 2000: The CA reported that she had discussed the worker's file with Board Medical Consultant, Dr. Germansky, stating: "He indicates to me the incident of Dec 13/99 shows there may have been some residual problem in the rt. shoulder as you would not expect a shoulder to dislocate if it was healthy and this appears related to [sic] the initial injury in this claim."
- January 26, 2000: In correspondence addressed to the Board, Dr. Sedran stated:

After attending strengthening exercises he returned to his previous level of function in November 1998. He did have a couple of occasions in the office when complained of discomfort and limitation in his shoulder but he was able to continue his usual activities.

In December 1999 he dislocated his shoulder once again during a non-work related activity. This dislocation occurred without trauma and during a light exercise routine. ...

These cases can be somewhat challenging to assess. There is a relationship between his current problem and his original work related shoulder dislocation. Dislocation of the shoulder results in laxity of the shoulder support muscles and predisposes the person to further dislocations. It is not possible that a normal functioning shoulder can dislocate after such a minor abduction force which was applied during his light exercise routine. The reason his shoulder dislocated with his minor force was because of the laxity and weakness that persisted in his shoulder support muscles after his initial dislocations. The

result is a somewhat unstable shoulder. One cannot diagnose an unstable shoulder until subsequent dislocation occurs. Unfortunately, in this patient he continues to be limited by his shoulder disfunction [sic] and has, once again, required physiotherapy. He may in fact [sic] require surgery to stabilize the shoulder musculature.

In summary, although there was no work related injury with this most recent dislocation and [the worker] had returned to his normal level of function approximately four months after his original dislocation, there is a relationship between his initial shoulder dislocation and his current shoulder problem.

- January 27, 2000: Dr. Brock reported that the worker's 1999 [should read "1998"] right shoulder dislocation "while at work ... was his first dislocation and predisposed him to possible future dislocations. His recent dislocation of the same shoulder is therefore related to the original injury."
- March 31, 2000: Memorandum No. 13, recorded a discussion between the CA and the worker's physiotherapist, in which the physiotherapist was reported to advise that the worker "... did a lot of kickboxing and she has recommended that he be careful with this activity as he will be prone to a re-dislocation."
- October 31, 2000: In a Progress Report, Dr. Sedran advised that the worker was experiencing mild shoulder pain on apprehension. "Still unable to do certain activities as the threat of dislocation exists." The diagnosis was "recurrent shoulder dislocation."
- March 30, 2001: An ambulance report stated that the worker "was walking down stairs and missed the last step striking his R elbow on a railing injuring his right shoulder."
- April 3, 2001: On referral from Dr. Sedran, the worker was assessed by orthopaedic surgeon, Dr. Holtby. He stated:

Since that episode which occurred in August, 1998, he has had a significant feeling of apprehension in the shoulder and has been unable to use his arm normally as a result. He describes three dislocations. After each episode, he has pain both with the use of his arm and at night for several days. He continues to be apprehensive even after resolution of pain with each episode. On examination, ... [h]e manages an overhead range of elevation of 150 degrees with some apprehension. ... He was quite apprehensive with all stress testing maneuvers...

[The worker] has very symptomatic instability of his shoulder. The only thing that will help this is surgical treatment.

- April 5, 2001: Dr. Sedran reported to the Board that following his first dislocation in 1998, the worker sustained two further dislocations following "minimal trauma." He stated that the worker "has had a recurrence of his shoulder problem which is "the result of a work related injury which occurred initially in August 1998."
- April 23, 2001: Board Medical Consultant, Dr. Germansky, provided the following opinion regarding the relationship between the worker's first and subsequent dislocation and his proposed right shoulder surgery. He stated:

I would agree with outside medical opinions relating the second dislocation to residual effects from the [sic] first noting no significant trauma had occurred. Having two dislocations it was quite likely his shoulder had become unstable. Although falling down the stairs is not trivial. I would accept the further dislocation under this claim. Any surgery done would be reasonably related to the initial and second events at minimum.

- May 17, 2001: Dr. Holtby performed an arthroscopic Bankart repair of the worker's right shoulder on a diagnosis of "recurrent dislocation right shoulder."
- December 18, 2001: Dr. Holtby reported to Dr. Sedran that assuming the worker's shoulder healed well, he could attempt to return to activities when he felt confident. He stated:

He is now at a stage where he can attempt to return to sporting activities if he feels confident enough. ...

I think there is still a possibility that his shoulder has not completely healed. If he does have further episode of instability or if his feelings of apprehension continue beyond the next 6 months, then we may have to consider repeat surgery. I do, however, feel that he will make a complete recovery eventually.
- October 19, 2007: Imaging of the worker's right shoulder revealed:

Glenohumeral alignment is within normal limits. There is a Hill-Sach's impaction type fracture at the posterior superior humeral head. In addition, there is some irregularity to the anterior inferior glenoid, consistent with a bony Bankart type lesion, however, I suspect this may be chronic long standing.

Cross sectional imaging would be of use in further assessment if the patient is chronically unstable.
- October 19, 2007: Emergency room records report the worker's attendance for a dislocation of his right shoulder due to a "sports injury." The corresponding ambulance report noted that the worker was working out and when he turned his foot slipped on a wet floor and he fell forward with his weight on his right forearm causing his right shoulder to dislocate.
- October 26, 2007: Imaging of the right shoulder identified:

A 12.0 x 12.0 mm triangular bony fragment is demonstrated at the anteroinferior aspect of the glenoid rim in keeping with a bony Bankart's lesion. There is also an angular deformity present in the posterosuperior humeral head in keeping with Hill-Sach's lesion.

[...]

Appearances are consistent with recent anterior dislocation of the shoulder with a Hill-Sach's lesion and a Bankart's lesion.
- November 19, 2007: The worker's physiotherapist diagnoses was "recurrent R GH dislocation."
- November 23, 2007: Dr. Sedran reported that the worker had a slip and fall on his elbow and had dislocated his right shoulder.
- January 18, 2008: Dr. Misra, orthopaedic specialist, reported that the worker had been symptom free for about six years:

...though he continued to have feelings of apprehension and minor instability during his routine daily activities during this period.

[...]

His x-rays done [in October 2007] and repeated today show some evidence of bony Bankart fracture and Hill-Sachs lesion.

We want to further assess this [Hill-Sach's] lesion and we have advised him to go for a CT scan. This will help us delineate the extent of the bony lesion and help us decide whether to go for an arthroscopic instability repair or open surgery.

Evidently this is a failed instability repair. Until the further decision is made [sic] regarding surgery, we have advised him to stay on modified duties and not to do any significant physical activity which might produce recurrent dislocations.

- January 22, 2008: In a report to the CA, Dr. Sedran stated:

The most recent shoulder dislocation, for which the reports of the 23rd of October was [sic] written, indicates a slip and fall injury causing the right shoulder dislocation. Specifically, his right arm slipped while doing a push-up and this resulted in the dislocation. Clearly, this was a fairly insignificant force, which caused the shoulder dislocation, indicating that there was a significant instability prior to this incident. This would suggest that the patient falls within that percentage of patients who fail the primary stabilization procedure.

There is absolutely no question that this patient's shoulder problem was precipitated by a work related injury. Subsequent dislocations occurred with minimal trauma are directly related to the instability which was caused by the original injury. Furthermore, the surgery was indicated to stabilize the shoulder, and once again was required as a result of the original injury. The evidence indicates to me that this is a compensable injury and an extension of his previous problems which have been well documented for the WSIB in the past.

- January 28, 2008: In a report to the CA, the worker's physiotherapist stated:

... I want to clarify the mechanism of injury for this patient. On November 12, 2007, [he] reported to me during his initial assessment that his R hand slipped on a wet spot on the floor while doing push ups. There was no report of new trauma to the R shoulder – so this is in keeping with a failed (unintelligible) surgical repair.

- April 5, 2008: Dr. Holtby advised Dr. Sedran that:

I have reviewed [the worker] after his CT scan of the right shoulder. This shows a large Hill-Sachs deformity of the humeral head. There are some post surgical changes in the inferior glenoid but I do not think there is a big glenoid bone defect.

- Dr. Holtby discussed humeral derotation osteotomy surgery with the worker along with "the potential risks of this more complicated stabilization surgery."

- August 26, 2008: In a further report addressed to the CA, Dr. Sedran stated:

In my most recent communication with you, in January of 2008, I advised you of my opinion that this patient's shoulder problems stem from the original dislocation, which was a well documented work related injury. Subsequent dislocations were a result of insignificant forces, which would not cause dislocation, under normal circumstances. In other words, the support structures in the shoulder were sufficiently damaged by the original dislocation that insignificant forces caused subsequent dislocations.

The patient underwent shoulder stabilization surgery by Dr. Holtby, an orthopaedic surgeon who specializes in shoulder surgery. The repair failed to stabilize the shoulder and further surgery will be required. I believe you are in possession of the reports of the orthopaedic surgeon.

Both he and I seem to agree that the shoulder problems were precipitated by his initial injury in the workplace. I understand this is in dispute. I would appreciate receiving any medical documentation from your medical consultant advising me why the medical assessments of me and Dr. Holtby have been contradicted.

- November 25, 2008: Dr. Holtby reported to Dr. Sedran that he was booking the worker for revision stabilization surgery on his right shoulder. He stated that he had “... advised him that in my opinion his subsequent problems are all related to that first dislocation which damage sufficient [sic] that he went on to have recurrent dislocations of his shoulder.”

(vi) The worker’s testimony

- [19] The worker testified that prior to his compensable right shoulder injury in 1998, he had been extremely active as a competitive kick boxer (having obtained his red sash), as the captain of his high school football team, and in other activities such as swimming and weight lifting. Prior to the 1998 injury, he had no problems with his right shoulder.
- [20] The worker described his 1998 compensable injury. At the time, he was working as a security officer at a shopping mall. On August 26, 1998, he was escorting an intoxicated patron from the mall to a subway station. The patron ran into a service station, picked up a squeegee, and threatened the worker with it. The worker blocked the squeegee and “took him down.” Both fell horizontally, with the worker landing on top of the patron. In the process, the worker struck his right elbow causing upward impact and the dislocation of his right shoulder. The worker testified that his pain was so great that he was immobilized. He was taken to hospital and was told the dislocation was severe and that he had to be sedated in order that the reduction of the shoulder could be performed. He was given morphine and later told that the reduction procedure had been very difficult and surgery had at one point been considered. In the end, however, the reduction was successful. The worker stated that for the next eight weeks he wore a sling, followed by a few months of physiotherapy treatment, that also included treatment for his elbow, which had become immobilized while he was wearing the sling.
- [21] Despite the treatment, the worker stated, his shoulder “never really recovered” and he was cognizant that the shoulder “was not what it used to be.” He continued to experience apprehension about the shoulder as with certain movements, he knew it would dislocate again. The worker testified that he was frightened by the possibility of dislocating the shoulder again, describing the experience as the “worst pain I ever had.” While he continued to involve himself in physical activities, he had to reduce their intensity. He ceased competitive kick boxing and only performed basic drills, which the worker likened to shadow boxing.
- [22] The worker described his second dislocation in December 1999 as occurring when he was shadow boxing. This did not involve a partner but included drills like throwing punches, kicking and practicing footwork. He equated this activity to aerobic kick boxing. As he threw his right hand straight-forward, his right shoulder dislocated and he fell to the floor of the boxing ring. In order that the ambulance attendants could have access to him, the ring had to be dissembled. The worker stated that the ambulance attendants had to seek authorization to give him pain medication. He was taken to hospital where the shoulder was reduced and he was given a sling and referred for physiotherapy. At that time, he told Dr. Sedran that he was experiencing joint pain and locking in his shoulder at night. Dr. Sedran offered to prescribe Percocet, but the worker explained that he did not want to “take that path” and instead relied on exercises. Dr. Sedran had also told him that now that he had had a second dislocation, he was more likely to more easily have it recur. The worker described his shoulder as having more laxity and being weaker at that time. Following this incident, the worker stated, he no longer participated in activities such as punching a bag, swimming or throwing a ball, as he once did.

- [23] The worker explained that at the time of his third dislocation in March 2001, he was working as a municipal licensing and standards office. He had come home from work and was descending stairs to his basement when he slipped and began to fall. He leaned on the railing with his right elbow which caused his shoulder to dislocate and then fell down the remaining stairs to the floor, again experiencing extreme pain. He was home alone and managed to drag himself to the telephone to call 911. As his doors were locked, fire fighters had to break a window to reach him. Compared to his condition in 1999, his shoulder following the 2001 incident was more lax and he experienced chronic pain with any movement, aching at night, and he would awake with apprehension pain. He also experienced a feeling of instability that suggested his shoulder “might go.” The worker stated that he returned to work with the municipality. While this job was not physical in nature, he did perform modified duties for a period of time.
- [24] It was also after the 2001 incident that he was referred to Dr. Holtby, who advised him that he needed surgery to the right shoulder to improve its stability. Dr. Sedran concurred in his need for surgery and told him that without it, the shoulder would get worse. The worker testified that following the procedure, his shoulder continued to be painful, he had a reduced range of motion in it due to the surgery, and he continued to experience feelings of instability. Drs. Holtby and Sedran advised him to continue to exercise and have the shoulder monitored. While he continued to work out, he did so at a reduced level, and he was no longer able to return to his prior level of activities.
- [25] Subsequent to the March 2001 injury, the worker applied to a transit authority as an officer. He underwent and completed the training, however, it was modified to take into account his shoulder. He stated that those trainees, who had prior injuries, had black tape placed over the injured area to avoid further injury.
- [26] The dislocation of October 2007 occurred when he was working out at his gym. The worker stated that he was on his knees with his arms extended in front of him. The floor was made of a smooth laminate and had been wetted by sweat, on which his hand may have slipped. The worker stated that he had 5 to 10% weight on his hands. As he attempted to push up, his right shoulder dislocated again. The worker stated that push-ups were, for him, a regular and normal part of his regime, equivalent to “riding a bike.” At the hospital he was again given a sling and sent to physiotherapy. At that time, Dr. Holtby told him he had a failed stability repair of the shoulder and that if he wanted to improve, he would need rotational osteotomy surgery, but the worker stated he did not proceed with this. His pain and instability in the shoulder continued to occur. Since then, he received treatment only from Dr. Sedran, did exercises and took pain killers to deal with his shoulder.
- [27] The worker testified that his shoulder had not dislocated since 2007. But because of the original injury in 1998 and the recurrences of the dislocation following it, his lifestyle and occupational choices were dramatically changed. He no longer involved himself in anything more than “light” physical activities. While his lifelong passion had always been to work in law enforcement, he suffered a \$40,000 pay cut when he left the profession and now worked in his job which required no physical activity. His home life was compromised by his inability to engage in activity with his children and he had suffered from depression.

(vii) Analysis

[28] In reaching a decision in this appeal, I have considered the law and Board Policy, the Case Record, and the worker's testimony and submissions. For the reasons that follow, I conclude that this appeal should be allowed.

(a) The recurrences

[29] There is no reference in the WSIA as to the definition of recurrence and how to determine whether a second injury is related to the original work accident. *Operational Policy Manual* ("OPM") Document No. 15-03-01, "Recurrences" is the applicable Board policy on recurrences. The policy provides that a decision-maker recognizes a recurrence when, in the absence of a new accident, there is either medical compatibility or a combination of medical compatibility and continuity with the original accident.

[30] Specifically, OPM Document No. 15-03-01 states, in part, the following:

Policy

A worker is entitled to benefits for a recurrence of a work-related injury or disease.

A recurrence may result from an insignificant new accident, or may arise when there is no new accident. To identify a recurrence, the WSIB must confirm that there is clinical compatibility between the original injury or disease and the current condition, or a combination of clinical compatibility and continuity.

Guidelines**Recognizing a recurrence****Clinical compatibility**

To establish clinical compatibility, a decision-maker compares the worker's current clinical condition to that following the initial accident. The decision-maker considers

- whether the parts of the body affected now are the same as, or related to, those affected initially
- whether the body functions affected are the same as those affected initially, and
- the degree to which body functions are affected now (as compared to the effect of the initial condition).

Similar clinical conditions indicate that the current problem or problems may be a result of the original injury, whereas dissimilar or unrelated clinical conditions indicate that there is no compatibility, and therefore no recurrence.

Continuity

To establish continuity (i.e. a connection between the original clinical condition and the most recent problem or problems), the decision-maker considers whether the worker has

- complained to supervisors, co-workers, or health care practitioners on an ongoing basis since the original injury
- demonstrates ongoing symptoms since the original injury
- required work restrictions or job modifications
- had ongoing treatment for the original condition, or

- experienced a lifestyle change since the original accident (e.g., has the worker become unable to participate in household duties, or social or recreational activities?).

[31] The essence of the argument submitted by the worker's representative, as I understand it, is that in context of OPM Document No. 15-03-01, none of the recurrences that followed the compensable injury of 1998 were of such significance as to constitute new accidents; rather they were minor in nature, and therefore should be considered recurrences as indicated by the policy. The representative also submitted that the worker's recurrences were clinically compatible with the compensable injury and continuity of the injury was demonstrated. The representative argued that the worker never fully recovered from his original injury, but rather, his condition worsened, as demonstrated by the medical reporting and the changes in the worker's personal and occupational circumstances. As a result of the original 1998 injury and the subsequent recurrences, there were changes in the shoulder including lesions, and the worker required surgery for the shoulder, which was ultimately categorized as a failed procedure. As such, his appeal should be allowed for the recurrences of 1999, 2001, and 2007 and the worker should be granted entitlement to a permanent impairment assessment of the right shoulder.

[32] OPM Document No. 15-03-01 provides that entitlement to a recurrence is based on the assessment of whether or not the recurrence is clinically compatible with the original injury. Put another way, it matters not whether the recurrence arose out of or occurred in the course of employment, or outside of employment, so long as the link can be made between the recurrence and the original injury. Thus, the fact that the worker's claimed recurrences of 1999, 2001 and 2007 occurred when the worker was not at work is not at issue. The issue is whether the incidents meet the criteria set out in OPM Document No. 15-03-01, namely, that the incidents of 1999, 2001 and 2007 were recurrences of his initial injury in August 1998.

[33] In addressing the question of what must be considered when determining medical compatibility/continuity, I have considered Tribunal decisions which have interpreted Board policy to mean that medical compatibility is the primary consideration. For examples see *Decision Nos. 1666/10, 2326/00, 1262/00, 709/04 and 2338 11*.

[34] In considering whether the worker's claimed recurrences were clinically compatible with the compensable right shoulder injury, it is evident that each instance involved re-injury to the right shoulder. Thus each of the recurrences involved the same body part and the same body function.

[35] With respect to the degree to which the worker's right shoulder function was affected as compared to the effect of his original right shoulder condition, the Board concluded that the worker had recovered from his initial right shoulder injury in November 2008 and as such, was not entitled to further benefits. The Board based this decision on the physiotherapist's January 12, 1999 report to the Criminal Injuries Review Board that the worker's period of disability stemmed from the date of his initial accident on August 26, 1998 to November 26, 1998. The Board also took into account Dr. Sedran's November 10, 1998 report that "I do anticipate a full recovery." Any acceptance of this conclusion would be incomplete, in my view, without a more detailed review of the relevant medical documentation.

[36] While in November 1998, Dr. Sedran indicated that he expected the worker to fully recover, he also noted that the worker continued to experience limitations in his shoulder movement and some apprehension. While in January 1999, the physiotherapist found the

worker's range of motion to be full, she noted that the worker had ongoing slight pain. It was also the worker's testimony that following his original injury and subsequent treatment, his shoulder "never really recovered." I also note that in his report of January 26, 2000, Dr. Sedran advised that while the worker had returned to his previous level of function, "on a couple of occasions in the office [he] complained of discomfort and limitation in his shoulder." Thus, despite predictions of full recovery, there is evidence that the worker's shoulder continued to be symptomatic.

[37] A deeper consideration of the extent of the worker's recovery from his 1998 dislocation also involves the exploration of the nature of the injury itself. Subsequent medical documentation referred consistently to the "recurrent" dislocations. (Dr. Chan, January 5, 2000; Dr. Sedran, January 18, 2000). Of more significance are the references to the relationship between an initial dislocation and a subsequent one. The physiotherapist noted in January 2000 that the worker had a previous dislocation and he was therefore "more prone to recurrence." Similarly, in January 2000, Dr. Brock, who attributed the second dislocation to the compensable first, was of the view that the worker's first dislocation "predisposed him to possible future dislocations." It is noteworthy in this regard that Board Medical Consultant, Dr. Germansky, opined to the CA in January 2000 that "the incident of Dec 13/99 shows there may have been some residual problem in the rt. shoulder as you would not expect a shoulder to dislocate if it was healthy and this appears related to [sic] the initial injury in this claim."

[38] Dr. Sedran elaborated on the correlation between the worker's first and second dislocations in his report of January 26, 2000, explaining:

These cases can be somewhat challenging to assess. There is a relationship between his current problem and his original work related shoulder dislocation. Dislocation of the shoulder results in laxity of the shoulder support muscles and predisposes the person to further dislocations. It is not possible that a normal functioning shoulder can dislocate after such a minor abduction force which was applied during his light exercise routine. The reason his shoulder dislocated with this minor force was because of the laxity and weakness that persisted in his shoulder support muscles after his initial dislocations. The result is a somewhat unstable shoulder. One cannot diagnose an unstable shoulder until subsequent dislocation occurs. [my emphasis]

[39] In her submissions, the worker's representative referred to the Tribunal's Medical Discussion Paper (MDP) entitled "Shoulder Injury and Disability."¹ The worker's representative specifically referred to the comment in the MDP that stated: "A first traumatic dislocation always precedes a recurrent shoulder dislocation." This is consistent with the medical reporting following the worker's 1999 recurrence, as noted above and is also consistent with the reporting that followed. In October 2000, Dr. Sedran noted that with activity "the threat of dislocation still exists." Thus, it is evident that while the Board concluded that the worker's shoulder condition had resolved in November 1998, I find that it did not as the worker was prone to further

¹ Dr. Hans K Uhtoff, Rev'd October 2010. The Tribunal's medical discussion papers deal with medical topics which frequently arise in appeals. They are written by independent experts who are recognized in their fields of specialization. The papers are not peer-reviewed publications, but are rather intended to provide parties and representatives with a broad, general overview of medical topics. A discussion paper is included in the case materials for an appeal when it appears that the paper may provide some relevant background to an issue in dispute. Medical discussion papers are also available on the Tribunal's website and in its Library. A Vice-Chair/Panel is not bound by any information or opinion expressed in a discussion paper, but may consider and rely on the general medical information provided by the paper. Every Tribunal decision must be based upon the facts of the particular appeal. It is always open to the parties to rely upon a discussion paper, or to distinguish or challenge it with other evidence.

dislocation as a result of his initial injury and that predisposition did not become evident until his second dislocation occurred.

[40] Thus, the nature of a dislocation shoulder injury may involve periods of relative stability, however, as in this case, the original injury caused weakness, laxity and instability so as to render the shoulder more readily prone to further dislocation.

[41] Mention was also made by Dr. Sedran about weakness, laxity and instability in the worker's shoulder. After his second dislocation in March 2001, on April 3, 2001, Dr. Holtby stated:

Since that episode which occurred in August, 1998, he has had a significant feeling of apprehension in the shoulder and has been unable to use his arm normally as a result. He describes three dislocations. After each episode, he has pain both with the use of his arm and at night for several days. He continues to be apprehensive even after resolution of pain with each episode. On examination, ... [h]e manages an overhead range of elevation of 150 degrees with some apprehension. ... He was quite apprehensive with all stress testing maneuvers...

[The worker] has very symptomatic instability of his shoulder. The only thing that will help this is surgical treatment.

[42] The MDP discusses this in the following way:

Shoulder Instability

The term is often used to describe a certain laxity of the capsule and components of the shoulder leading to subluxation. It may result from a first time dislocation, an injury. In the absence such an injury, the condition cannot be accepted as work-related. Patients complain about a feeling of insecurity while attempting certain movements.

[43] In April 2001, Dr. Germansky also noted that the worker likely suffered from shoulder instability, stating that:

I would agree with outside medical opinions relating the second dislocation to residual effects from he [sic] first noting no significant trauma had occurred. Having two dislocations it was quite likely his shoulder had become unstable. ... I would accept the further dislocation under this claim. Any surgery done would be reasonably related to the initial and second events at minimum.

[44] The medical reporting, consistent with the description in the MDP, establishes that a recurrent dislocation cannot occur without an initial injury. As well, it is evident that a shoulder dislocation can result in weakening, laxity and instability of the shoulder generating an increased possibility of future dislocation(s). This, in my view, dispels any suggestion that the worker had fully recovered in November 1998 from his original injury and that his second dislocation was unrelated.

[45] As the medical reporting indicates, following the second dislocation there is evidence that the degree of condition deteriorated as compared to the effects of the initial injury. Moreover, following the third dislocation of March 2001 Dr. Holtby performed an arthroscopic Bankart repair of the worker's right shoulder in May 2001.

[46] The worker's condition remained apparently stable until he suffered his fourth dislocation in October 2007. Imaging on October 26, 2007 identified the presence of a bony Bankart's lesion and deformity of the posterosuperior humeral head in keeping with a Hill-Sach's lesion. Noting this in his assessment of January 2008, Dr. Misra stated: "Evidently this is a failed instability

repair,” advising the worker against any significant physical activity which “might produce recurrent dislocations.” Thus, by the time of the October 2007 dislocation, the worker had additionally suffered from a Hill-Sach’s lesion in the shoulder. This is described by the MDP in the following manner. “A shoulder dislocation is sometimes accompanied by a compression fracture of the humeral head, a lesion known as Hill-Sach’s compression fracture of the humeral head.” This is consistent with the findings of the CT scan of the worker’s right shoulder. Dr. Holtby further confirmed this in his report of April 5, 2008 addressed to Dr. Sedran, when he stated:

I have reviewed [the worker] after his CT scan of the right shoulder. This shows a large Hill-Sachs deformity of the humeral head. There are some post surgical changes in the inferior glenoid but I do not think there is a big glenoid bone defect.

[47] The final reporting of Drs. Sedran and Holtby lend further credence to the correlation between the worker’s initial injury and his subsequent recurrences as well as to the progressive decline in the degree of his condition as compared to the original injury. Dr. Sedran advised the CA on August 26, 2008 that:

In my most recent communication with you, in January of 2008, I advised you of my opinion that this patient’s shoulder problems stem from the original dislocation, which was a well documented work related injury. Subsequent dislocations were a result of insignificant forces, which would not cause dislocation, under normal circumstances. In other words, the support structures in the shoulder were sufficiently damaged by the original dislocation that insignificant forces caused subsequent dislocations.

The patient underwent shoulder stabilization surgery by Dr. Holtby, an orthopaedic surgeon who specializes in shoulder surgery. The repair failed to stabilize the shoulder and further surgery will be required. I believe you are in possession of the reports of the orthopaedic surgeon.

Both he and I seem to agree that the shoulder problems were precipitated by his initial injury in the workplace. ...

[48] Despite his earlier predictions that the worker would recover, on November 25, 2008, Dr. Holtby reported to Dr. Sedran that he was booking the worker for further revision stabilization surgery on his right shoulder. He stated that he had “... advised him that in my opinion his subsequent problems are all related to that first dislocation which damage sufficient [sic] that he went on to have recurrent dislocations of his shoulder.”

[49] Thus, in my view, despite periods of relative stability, the worker nonetheless continued to suffer from advancing shoulder instability, weakness, laxity and apprehension in his shoulder that rendered him prone to the further three recurrences of shoulder dislocation he experienced in 1999, 2001 and 2007. As well, following his 2001 recurrence, it was necessary for the worker to undergo stabilization surgery. This was deemed to be a failed procedure following the 2007 dislocation and further, the worker was diagnosed with the Hill-Sach’s lesion, leading Dr. Holtby to conclude that further stabilization revision surgery was necessary. Similarly, in January 2008, upon referring to the worker’s 2007 dislocation, Dr. Sedran’s stated opinion was that:

[c]learly, this was a fairly insignificant force, which caused the shoulder dislocation, indicating that there was a significant instability prior to this incident. This would suggest that the patient falls within that percentage of patients who fail the primary stabilization procedure.

[50] Additionally, in January 2008, the worker's physiotherapist, also referring to the 2007 dislocation advised the CA:

On November 12, 2007, [he] reported to me during his initial assessment that his R hand slipped on a wet spot on the floor while doing push ups. There was no report of new trauma to the R shoulder – so this is in keeping with a failed (unintelligible) surgical repair.

[51] I further note that the MDP states that “[i]f dislocation occurs without a significant trauma after surgery, one must conclude that the surgical repair has not been successful.”² Consistent with the requirements of OPM Document No. 15-03-01, this evidence establishes clinical compatibility between the worker's current condition and his initial injury. I would add that Dr. Sedran, Dr. Holtby, Dr. Chan, the physiotherapist, as well as Dr. Germansky were all consistent in their opinions that the worker had suffered recurrences of his initial 1998 injury.

[52] I do not overlook Board's concern that the worker's recurrences were more than insignificant and therefore not recurrences but were new accidents. The determination of this issue depends on the assessment of the mechanics of each of the recurrences, surrounding which, there is some confusion in the file.

[53] At the outset, I make the general observation that prior to his compensable injury of August 1998, the worker led a very active physical lifestyle. He was a football player, an accomplished competitive kick boxer and involved himself in activities including swimming and weight lifting. There is no dispute that his original injury was traumatic as occurring when he wrestled an intoxicated patron to the ground while performing the duties of a security officer.

[54] The recurrence of December 1999 is generally described as having happened when the worker threw a punch. I note that the emergency room report of December 13, 1999 described the injury as occurring while the worker was “kick boxing – punched and dislocated.” While this may be interpreted as involving the worker in a fight with another individual, a closer review suggests otherwise. The worker testified that his shoulder dislocation resulted in the process of shadow boxing, or performing kick boxing aerobics. This is consistent with the more detailed report of Dr. Sedran in January 2000, who stated: “In December 1999 he dislocated his shoulder once again during a non-work related activity. This dislocation occurred without trauma and during a light exercise routine.” Similarly, Dr. Chan described the incident as: “Last week he was at the gym and was simply throwing a punch with no contact and dislocated the shoulder again.” Given that the worker had by this time ceased competitive kick boxing, and given that he was highly accustomed to the aerobic aspect of his sport, I find that the effort of throwing a punch without contact an insignificant and non-traumatic event and not a new accident.

[55] The March 2001 incident was seen by Dr. Germansky as the worker “falling down the stairs” which was “not trivial.” The worker testified that when he slipped on the stairs, he first leaned his elbow on the railing before actually falling. Consistent with this, the ambulance report at the time stated that the worker “was walking down stairs and missed the last step striking his R elbow on a railing injuring his right shoulder.” In my view, had the worker fallen on his shoulder, this incident could have been categorized as traumatic, however, the act of leaning on a railing with an elbow to stabilize a fall was minor when considering that the consequence was to dislocate the shoulder. I note as well that in referring to both the 1999 and 2001 dislocations, in April 2001, Dr. Sedran described the incidents as “minor traumas.” Also, despite his reference to

² I deal with the issue of the significance of the worker's 1999, 2001 and 2007 accidents below.

falling down stairs, Dr. Germansky stated: “I agree with outlined medical opinions regarding the second dislocation to residual effects from the first noting no significant trauma had occurred.” Thus, I find that the March 2001 incident was not a significant new accident.

[56] The March 2007 recurrence was described as occurring while the worker was doing a push up. The worker explained that he was on his knees, bearing most of his weight, when his hand slipped on the wet floor, resulting in the dislocation. Dr. Sedran advised the CA on January 22, 2008 that this was not a slip and fall, but rather “his right arm slipped while doing a push up and “[c]learly, this was a fairly insignificant force, which caused the shoulder dislocation indicating that there was a significant instability prior to this incident.” Similarly, in her January 28, 2008 report to the CA, the worker’s physiotherapist stated:

... I want to clarify the mechanism of injury for this patient. On November 12, 2007, [he] reported to me during his initial assessment that his R hand slipped on a wet spot on the floor while doing push ups. There was no report of new trauma to the R shoulder – so this is in keeping with a failed (unintelligible) surgical repair.

[57] The reports of Dr. Sedran and the physiotherapist’s are consistent with the worker’s description of the mechanics of the 2007 recurrence. Noting once again that this exercise was one in which the worker was highly accustomed to perform, I find that it was an insignificant incident resulting in yet a third dislocation of his right shoulder.

[58] I would add that I had the benefit of hearing the worker’s testimony. I found it to be consistent with the record and straight-forward, unembellished and genuine.

[59] I would also conclude that in addition to compatibility, continuity of complaint is also established. As such, the worker has entitlement for his right shoulder condition beyond November 26, 1998. As previously stated, notwithstanding periods of stability, the nature of an initial shoulder dislocation is to render the shoulder more vulnerable to future dislocations. In this case, the worker suffered three recurrences of his shoulder injury of August 1998. Subsequent to the initial injury, he continued to complain about his shoulder to his doctors and the medical evidence confirms his ongoing right shoulder symptoms. He was required to work with restrictions and he experienced lifestyle changes following his original accident. He also underwent ongoing treatment for the shoulder, which deteriorated following his injury in 1998, including the surgery of 2001 and following. This demonstrates continuity between the worker’s original clinical condition and his subsequent right shoulder problems.

[60] I conclude, therefore, that worker suffered recurrent shoulder dislocations in 1999, 2001 and 2007 as the result of insignificant new accidents and that there was clinical compatibility and continuity between the new accidents and his original injury. The appeal is allowed for ongoing entitlement for the worker’s right shoulder from November 26, 1998, including his recurrences in 1999, 2001 and 2007.

(b) Permanent impairment

[61] As noted above, entitlement for a permanent impairment is conditioned on the finding that the impairment exists beyond the point of MMR. OPM Document No. 11-01-05, “Determining Permanent Impairment,” defines MMR as follows:

Workers reach ... (MMR) when they have reached a plateau in their recovery and it is not likely that there will be any significant improvement in their medical impairment.

[62]

In this case, the worker suffered two recurrences of his right shoulder injury in 1999 and 2001. He underwent an arthroscopic Bankart repair on May 17, 2001. That surgery was ultimately determined to have been a “failed instability repair” by Dr. Misra in January 2008. That opinion was echoed as well by Dr. Sedran and the worker’s physiotherapist. That seven years had passed before this conclusion was reached, in my view, was only due to the nature of the worker’s condition. As was noted above, the failure of the procedure was only identifiable after a further dislocation following the surgery. As such, I conclude that the worker reached MMR as of May 17, 2001 and he is, therefore, entitled to a permanent impairment assessment of his right shoulder.

DISPOSITION

[63] The appeal is allowed.

1. The worker did not recover from his compensable shoulder injury of August 26, 1998.
2. The worker has ongoing entitlement beyond November 1998, including the recurrences of December 13, 1999, March 30, 2001 and October 19, 2007.
3. The worker has entitlement to a permanent impairment assessment for the right shoulder, with an MMR date of May 17, 2001.

[64] The nature and duration of the benefits flowing from this decision are returned to the Board for further adjudication, subject to the usual rights of appeal.

DATED: August 20, 2015

SIGNED: W. Sutton